

3

Name	narendra
Age / Sex	24 years / Male
Contact	9966288487
Referral Doctor	shiva raju
This Section	An abridged abstract. Not a Clinical Laboratory Report

INVESTIGATION	YOUR CURRENT VISIT		FROM YOUR PREVIOUS 3 VISITS ^a
	07-Aug-26	BRI ^b	
Creatinine - Serum	1.22	0.67-1.17	
Method: Jaffe kinetic IDMS Traceable			

NOTES

- From previous 3 visits in preceding 2 years. If your past results are not mentioned, please contact us at 040-6700 6700.
- The dates mentioned in the above table are the corresponding Order Date and may differ from the Sample Drawn Date. Please refer to the relevant Clinical Laboratory Report.
- BRI = Biological Reference Interval. Depending on the test / parameter, the BRI may differ on account of gender and age. For the sake of brevity, the units have not been mentioned in this table. Please refer to the Clinical Laboratory Report for details.

Note: Please contact us for possible remedial action if test results are unexpected.

 Abnormal  * Critical

Screening & Vaccination Recommendations

Grade A	Colorectal Cancer, HIV, Hypertension Grade-A: As categorized and recommended by the US Preventive Services Task Force. There is high certainty that the net benefit is substantial
Annual	Alcohol Abuse (Alcohol Level Serum, GTT), Hepatitis C (Hepatitis C Virus-HCV Antibody ELISA), HIV (HIV 1 & 2 Antibody ELISA), Tobacco Use (Urine Cotinine Nicotine Test), Hepatitis B (Hepatitis B Surface Antigen-HBsAg ELISA)
Periodic: every 2y	Hypertension (Hypertension Profile), Obesity (MDx Weight Watch Advanced), USG-Liver, USG-Abdomen, X Ray-Chest, ECG, 2D Echo, BMD
Periodic: every 3y	Diabetes (HbA1c), MRI-Brain, MRI-Spine, CT-Abdomen, CT-Chest
Periodic: every 5y	Colorectal Cancer (Colonoscopy, MSI Testing), Hyperlipidemia (Lipid Profile), Lung Cancer
Vaccinations: Important	Hepatitis B, Human Papillomavirus (HPV), Influenza, Tetanus, Diphtheria, Acellular Pertussis (Tdap and Td)

Important Disclaimer

Please consult a Registered Medical Practitioner for review and consultation, before proceeding on the above assessment and recommendations

Name	narendra	Order	PTGOC2700486616	
Age / Sex	24 years / Male	Sample Drawn	07-Aug-26 / 07:43 AM	
Contact	9966288487	Sample Accepted	07-Aug-26 / 10:56 AM	
Collection Centre	MHTGHYD00023	Sample Reported	07-Aug-26 / 12:17 PM	
Referral Doctor	shiva raju	Report Status	Final	

Creatinine, Serum

SampleType: Serum

Department of Clinical Biochemistry

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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Creatinine - Serum**1.22**

mg/dL

0.67-1.17

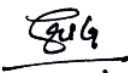
Method: Jaffe kinetic IDMS Traceable

Clinical Significance: Creatinine

- Measurements of creatinine are used in the diagnosis and treatment of renal disease and prove useful in the evaluation of kidney glomerular function and in monitoring renal dialysis.
- Serum level is not sensitive to early renal damage and responds more slowly than blood urea nitrogen (BUN) to haemodialysis during treatment of renal failure.
- **Both serum creatinine and BUN are used to differentiate prerenal and postrenal (obstructive) azotemia.**
- An increase in serum BUN without concomitant increase of serum creatinine is key to identifying prerenal azotemia.
- In post renal conditions where obstruction to the flow of urine is present e.g. malignancy, nephrolithiasis and prostatism, both the plasma creatinine and urea levels will be increased; in these situations the rise is disproportionately greater for BUN




M Nageshwar Rao
 Lab Manager


Dr G Srinivas
 Director-Lab Services &
 Chief Pathologist
 Regd no:
 TSMC-49913

END OF THE REPORT

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Uric Acid, Serum

SampleType: Serum

Department of Clinical Biochemistry

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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Uric Acid - Serum**8.08**

mg/dL

3.5-7.2

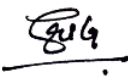
Method: Uricase POD

Clinical Significance: Uric acid

- **Primary hyperuricaemia** is associated with gout, Lesch-Nyhan syndrome, Kelley Seegmiller syndrome and increased phosphoribosyl pyrophosphate synthase activity.
- **Secondary hyperuricaemia** is associated with numerous conditions including renal insufficiency, myeloproliferative diseases, haemolytic diseases, psoriasis, polycythemia vera, type I glycogen storage disease, excess alcohol consumption, lead intoxication, a purine-rich diet, fasting, starvation and chemotherapy.
- **Hypouricaemia** may also be due to increased renal uric acid excretion, which may occur in malignant diseases, AIDS, Fanconi syndrome, diabetes mellitus, severe burns and hypereosinophilic syndrome.
- Quantitation of urinary uric acid excretion may assist in the selection of appropriate treatment for hyperuricaemia, providing an indication of whether patients should be treated with uricosuric drugs to enhance renal excretion, or allopurinol to suppress purine synthesis.




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Complete Urine Examination (CUE)

SampleType: URINE

Department of Clinical Pathology

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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PHYSICAL EXAMINATION

Colour	Pale Yellow	Pale Yellow
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Method: Physical

Appearance	Clear	Clear
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Method: Physical

CHEMICAL EXAMINATION

Reaction and PH	5.0	4.6 - 8
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Method: Double Indicator

Specific Gravity	1.016	1.003 - 1.035
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Method: Bromothymol Blue

Glucose	Negative	Negative
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Method: Glucose oxidase Peroxidase

Protein	1+	Negative
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Method: Protein error of Indicator

Blood	Negative	Negative
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Method: Peroxidase like activity

Ketones	Negative	Negative
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Method: Sodium Nitroprusside

Bilirubin	Negative	Negative
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Method: Dichloroaniline diazonium

Leukocytes	Negative	Negative
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Method: 3hydroxy 5phenylpyrrole diazonium

Urobilinogen	Normal	Normal
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Method: Ehrlichs reaction

Nitrites	Negative	Negative
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Method: Diazonium

Epithelial cells (EC)	2.0	/HPF	2-5
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RBC	0.0	/HPF	Nil
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WBC (Pus cells)	1.3	/HPF	1-2
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 * Critical

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Complete Urine Examination (CUE)

SampleType: URINE

Department of Clinical Pathology

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
---------------	--------	-------	-------------------------------

H Laxminarayana
Verified byDr Rahmath Unissa
Consultant Pathologist
Regd no:
APMC/FMR/93751

END OF THE REPORT

Conditions of Reporting

1. Laboratory reports will aid in diagnosis of clinical conditions in conjunction with clinical signs, symptoms and related investigations. They are best interpreted by qualified medical professionals who understand reporting units, reference ranges and limitations of technologies and their correlation with other clinical findings.
2. The interpretations provided by MedPlus are for the guidance of patients and referring doctors. MedPlus nor its affiliates assume any liability or responsibility for any damage of any nature whatsoever that may be incurred in any person as a result of the use of the information provided in the report.
3. It is presumed that the test(s) performed are, on the specimen(s)/sample(s) belonging to the patient named or identified and the verification of the particulars have been carried out by the patient or his/her representative at the point of generation of the said specimen(s) or sample(s).
4. The results of tests may vary from lab to lab and also from time to time for the same parameters for the same patient. Assays are performed with reasonable care and in accordance with standard procedures. The reported results are dependent on individual assay methods, equipment used, method specificity, sensitivity, drug interaction and the quality of the specimen(s)/samples(s) received.
5. Should the results indicate unexpected abnormality, the same should be reconfirmed after appropriate clinical correlation.
6. Histopathology specimen(s)/sample(s) will be preserved for one month from the date of testing and slides/reports will be preserved for five years. Other clinical specimen(s)/sample(s) will be discarded after seven days from the date of testing, unless otherwise specified by the client. Such preservation shall be subject to sample integrity.
7. Preliminary Report, if any indicates that the results are primary and they are yet to be reported for one or more of the tests, or else, as in case with many microbiology test, a "final" culture, identification or drug susceptibility result might be pending. When all results are available the "Preliminary report" will be replaced by "Final Report". Client shall rely only on the final report.
8. This report is not valid for Medico-legal purposes.
9. Tests are performed as per the test schedule in the test listing. In unforeseen circumstances such as non-availability of relevant kits, instrument breakdown, natural calamities etc, tests may not be reported as per schedule.
10. The sex of the foetus will not be revealed as per the prenatal diagnostic techniques (PNDT) Act, 1994.
11. All queries pertaining to this report should be directed to MedPlus Health Services Limited
12. All investigations are limited by the sensitivity and specificity of the assay and the condition of the specimen received by the laboratory. Assay result should be interpreted only in the context of other clinical findings and the clinical status of the patient.
13. Partial reproduction of this report is not permitted.

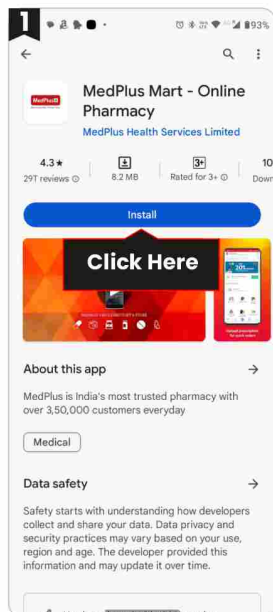
Accreditation

NABL accreditation signifies that a laboratory meets international quality standards and is competent to provide accurate and reliable test results. NABL accreditation is awarded to those laboratory's whose testing and calibration services are of high quality.

IMPORTANT

You can ask for a copy of the NABL certification of MedPlus Diagnostics. Please email: wecare@medplusindia.com

Follow these steps to see all results from last 1 year, in one table

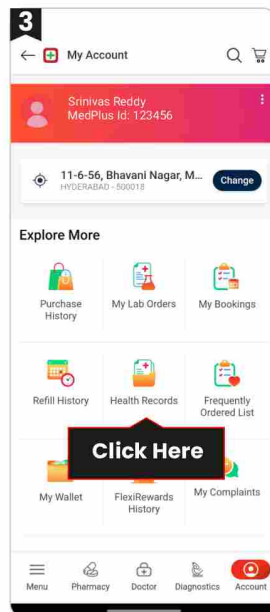


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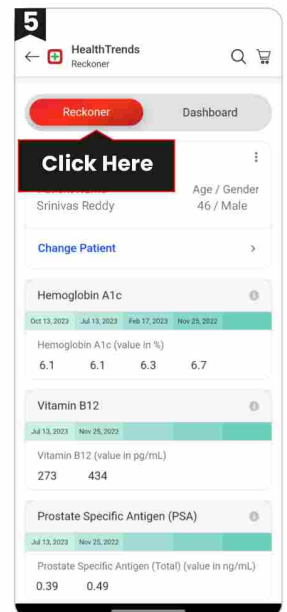


In this section, "Explore More", click on "Health Records".



In this section you can access visit/ test reports.

In the band on top, click on "Health Trends".



Click on "Reckoner", to see the test-wise tabulation of your lab results, from past and current visits.

Vaccines at MedPlus: One shot today, Smile away

Vaccine	Who should take it?	You should know	If already immunized
Diphtheria Toxoid	For all except if contraindicated	Mortality rate for untreated diphtheria can be as high as 10% to 20%	Age grp: 18 to 64yrs Booster: Once every 10yrs till the age of 65yrs
HPV	For all except if contraindicated (both male and female)	HPV can lead to cancer (cervical cancer being the most common)	No booster needed
Influenza	For all (especially for individuals at high risk)	Pneumonia is one of the common complications of influenza	Annual dose
Tetanus	For all except if contraindicated	Mortality rate for unvaccinated or poorly vaccinated population is 30%	Booster dose once every 10yrs
Typhoid	For all except if contraindicated	Mortality rate for untreated typhoid fever is 10-30%	Booster dose once every 3yrs
Varicella	For all who are not immune	10% of individuals with Varicella experience secondary bacterial infections	Recommended only if titers are inadequate
Zoster	For all > 50yrs or individuals at high risk		No booster needed
Pneumococcal (PCV15, PCV20, PPSV23)	For all > 65yrs or individuals at high risk		No booster needed
Anti-Rabies	Individuals at high-risk (like Vet physician and pet owners) or post-exposure		2 doses at Day 0 and Day 3 with no immunoglobulin when exposed
Hepatitis A	Individuals at high risk		No booster needed
Hepatitis B	Individuals at risk	Hepatitis B virus is a leading cause of liver cancer globally	Recommended only if titers are inadequate
Measles	Individuals at high risk		Single dose (recommended only if titers are inadequate)
Meningococcal (Grp A, C, Y, W)	Individuals at high risk		Booster dose once every 5yrs

Feedback and Result Queries

 wecare@medplusindia.com

 medplusmart.com

 +91 40 6700 6700

RECOMMENDED LIFESTYLE-DISEASES ASSESSMENT TESTS

Dr. Venkat Nageshwar Goud

MBBS, DMRD

HOD - Radiology & Chief Radiologist



1

ESSENTIAL INSIGHT INTO YOUR BODY COMPOSITION

BODY FAT ANALYSIS (DEXA)

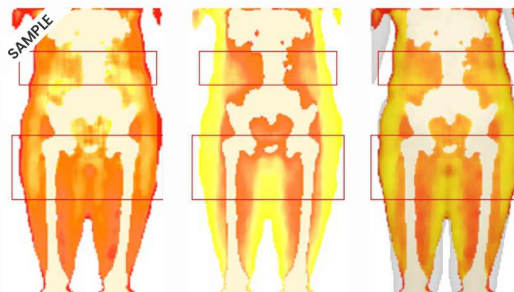
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Our DEXA machines use very low levels of x-rays to separately measure Fat mass, Lean mass and Bone mass.

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The progress of your program can also be measured by regularly monitoring Fat mass, Lean mass and Bone mass.



→ ANDROID

→ GYNOID

11.2
kgs

FAT MASS

14.9
kgs

LEAN MASS

0.4
kgs

BONE MINERAL

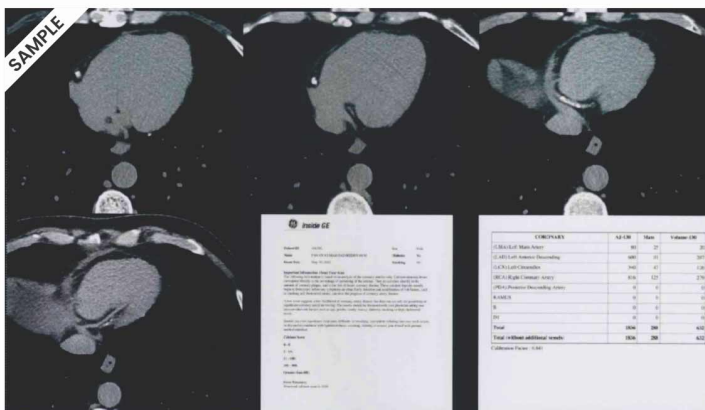
Android refers to the midsection of the torso, in the waist area, under the rib cage

Gynoid refers to the lower torso, in the hip area, down to the top of the thighs

FOR ANDROID AND GYNOID REGIONS

2

BEST VALUE, IF YOU ARE > 40 YEARS



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